

Management of patients with Gastrointestinal diseases

Dr . Bardies shaban





الحديث الجانبي يشتت الانتباه



من فضلك اغلق الهاتف

Peptic ulcer

cholecystitis

Hepatitis

Liver cirrhosis

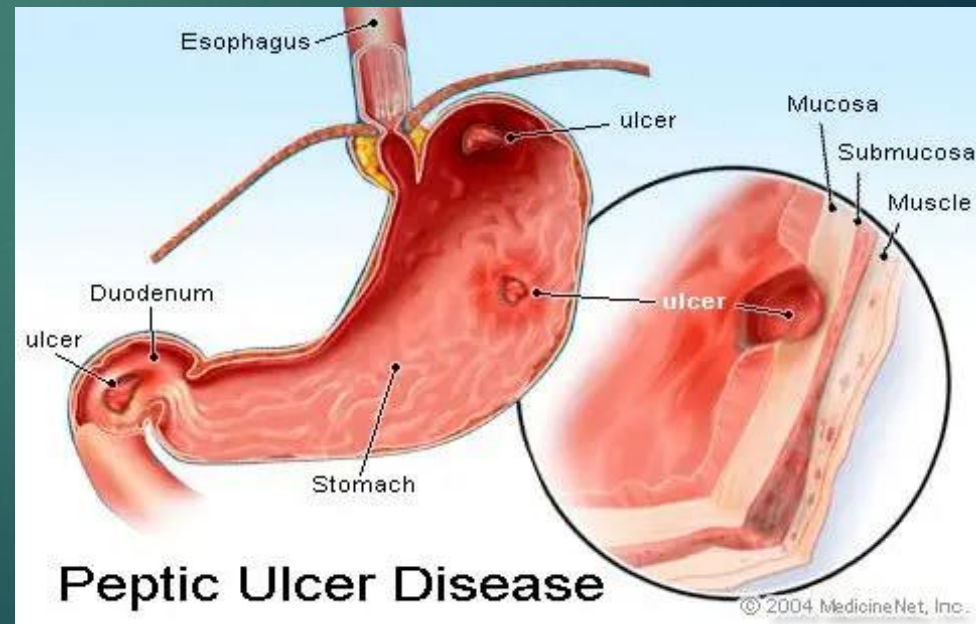
Esophageal varices



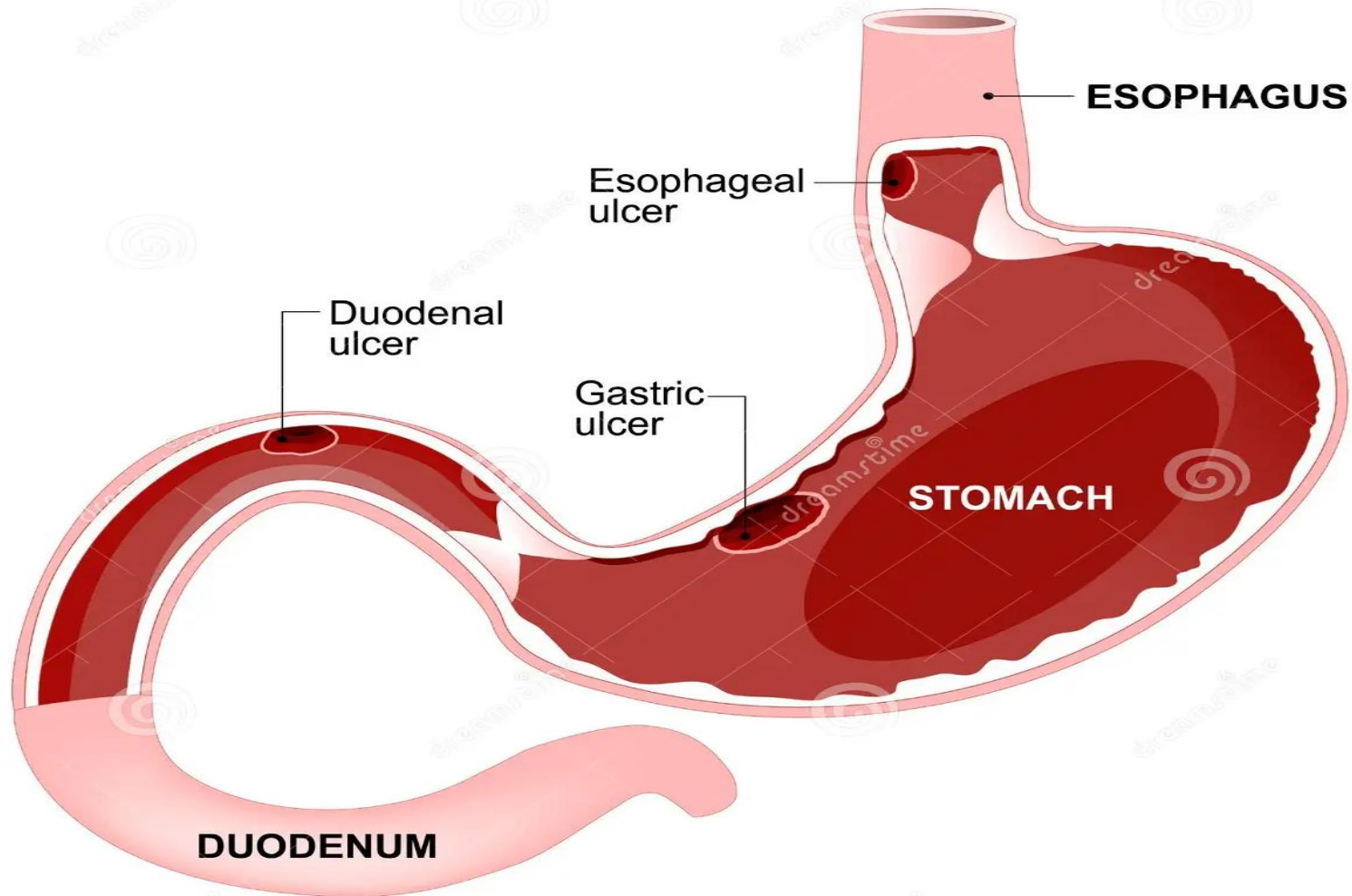
1- Peptic ulcer

- ◉ Definition
- ◉ Causes
- ◉ Manifestations
- ◉ Diagnosis
- ◉ Complications
- ◉ Managements

Definition: Break in the continuity of the esophageal, gastric, or duodenal mucosa. It occurs in any part of gastrointestinal tract that comes in contact with hydrochloric acid and pepsin .



Location of peptic ulcer



Causes

- ▶ A major causative factor (60% of gastric and up to 90% of duodenal ulcers) is chronic inflammation due to Helicobacter pylori
- ▶ Emotional stress
- ▶ Drugs such as Aspirin and other NSAIDs
- ▶ Chronic gastritis
- ▶ Cigarettes smoking and using alcohol
- ▶ Overuse of laxatives is also known to cause peptic ulcers.
- ▶ A family history is often present in duodenal ulcers.

Peptic Ulcer

What are the risk
factors for peptic
ulcer?



Smoking



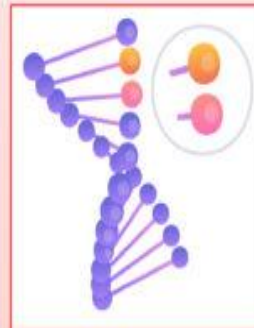
Excess consumption
of alcohol



Spicy food



Stress



Family history of
Multiple Endocrine
Neoplasia



High dose
corticosteroids

Helicobacter
Pylori
bacteria

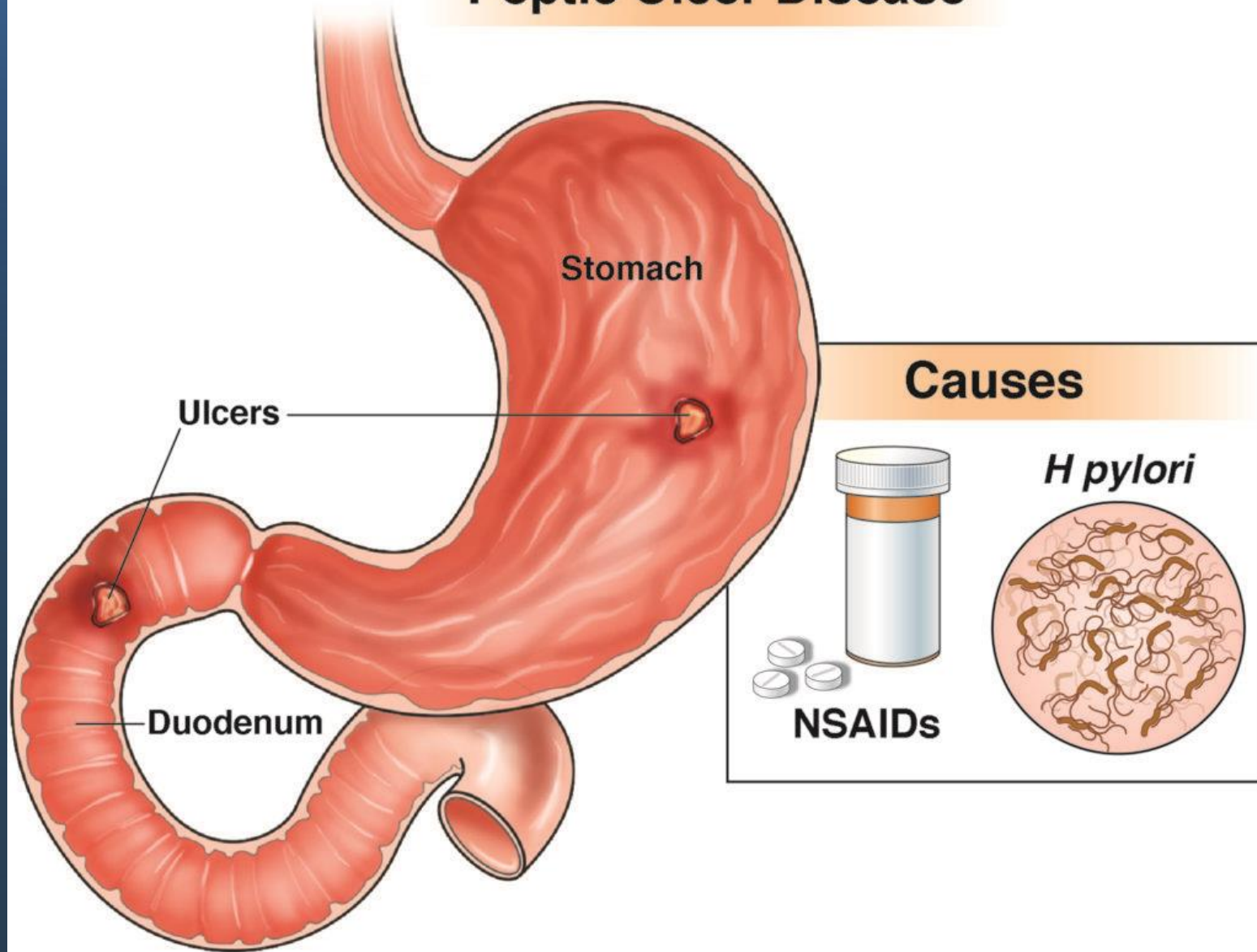


Stomach
ulcer

Duodenal
ulcer



Peptic Ulcer Disease



Major Symptoms of Ulcer



*Burning Stomach
Pain*



Bloating



Heartburn



Nausea



Vomiting



*Dark, Tarry
Stools*



*Unexplained
Weight Loss*



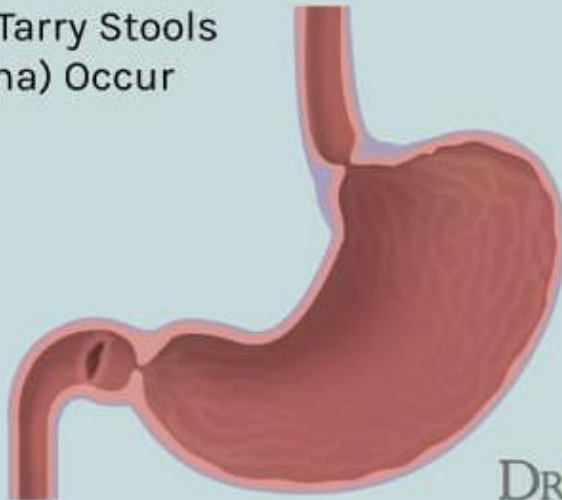
Fatigue



Indigestion

Duodenal Ulcers

- Pain relieved by meal.
- Occurs 2-3 hrs after meal.
- Most common type.
- Dark, Tarry Stools (Melena) Occur



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Gastric Ulcers

- Pain increased by meal.
- Occurs 30m to 1hr after meal.
- Not as common.
- Vomiting occurs.



Duodenal Ulcer

Gastric ulcer

Age	Any age specially 30-40	middle age 50-60
Sex	More in male	More in female
Occupation	Stress job eg. Manager	Same
Pain	Epigastric , discomfort	Epi. Can radiate to back
Onset	2-3 hours after eating & midnight	Immediately after eating
Agg.by	Hunger	Eating

Assessment and diagnostic studies :

- ❖ Endoscope: By direct visual identification, the location and severity of an ulcer can be described.
- ❖ Culture
- ❖ Biopsy
- ❖ Stool antigen test.

Complications of Peptic ulcer disease

Bleeding



Perforation



Obstruction



Causes: *Helicobacter pylori* in 70-9

Management

Medical :

- **Antacids** ; neutralize gastric acid that reduce gastric acid.
- **Antibiotics**
- In the absence of H. pylori, long-term higher dose of **Proton Pump Inhibitors** (PPIs) is often used

Nursing process

1- Assessment :

- Complete assessment of **pain**.
- Ask patient to list usual **food intake for 72 hours** to describe food habits as use of spicy foods, and caffeinated beverages and coffee.
- Assess **vital signs** and report **tachycardia** and **hypotension** as they may indicate **anemia** from GIT bleeding.

2- Nursing diagnoses :

- **Acute Pain** related to increase gastric acid secretion
- **Imbalanced nutrition** related to changes in diet
- **Anxiety** related to coping with an acute disease
- **Knowledge deficit** related to prevention of symptoms and management of the disease.
- **High risk for bleeding** as complications of peptic ulcer

Nursing process

3- Nursing interventions :

goals : **Relieve pain**

- Avoid aspirin, foods and beverages that contain caffeine as coffee and tea.
- Follow regular meals.
- Administer medication as prescribed

Nursing process

Dietary modification *to avoid over secretion of acid by:*

- ✓ Avoid alcohol, coffee, smoking and other caffeinated beverage.
- ✓ Regular eating pattern to neutralize acid.
- ✓ Small frequent diet.
- ✓ Eat foods that can be tolerated and avoid those that produce pain.

Food to Avoid if you have Stomach Ulcer

Alcohol



Milk and Milk Products



Coffee



Red Meat



Carbonated Drinks



Salt & Salted food



PEPTIC ULCER DIET:

BEST AND WORST FOODS FOR STOMACH HEALTH



Nursing process

Dietary modification

- During meal: semi fowler position
- After meal rest for 20-30 minutes.
- Fluids are discouraged with meals but may be given 1 hour before or 1 hour after mealtime.
- Dietary supplements as vitamin B12 and iron may be prescribed

Nursing process

goals : ***Reduce anxiety***

- Allow patient to express feeling.
- Explanation about diagnosis, treatment and medication.
- Good interaction between patient and nurse.
- Encourage the family to participate in patient care and emotional support

Nursing process

goals : ***Patient teaching***

- Instruct patient about importance of continuing taking medication even after signs and symptoms have decreased.
- Avoid smoking
- Rest and stress reduction.
- Awareness of complications
- Follow up care

Nursing process

goals : ***Monitoring and managing complications***

- Assess patient for **bleeding**
- Monitor **vital signs**
- Monitor **hemoglobin**
- Record hourly **urinary output** to detect anuria or oliguria.

Nursing process

goals : ***Monitoring and managing complications***

- Rapidly **replace** the blood lost.
- Insert **urinary catheter** to monitor urinary out put
- Administer **O₂** therapy.
- Place patient in **supine position** with legs elevated to prevent hypotension.

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Summary



- List 3 risk factors of peptic ulcer
- List 4 manifestations of peptic ulcer.
- Differentiate between gastric and duodenal ulcer



Thank
you